INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Dacomitinib monotherapy

Understand your treatment. Know what to expect. Keep your plan to hand.

Treatment pattern
Ongoing
Follow the prescribed daily plan

Regimen source
NCCP 00565
Version 3

> Daily oral treatment without a rest week

Inside your handbook

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My name _____

Hospital / oncology team _____

> Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

Understanding your treatment

Dacomitinib monotherapy · National Cancer Control Programme (NCCP) 00565

How this medicine supports your treatment

Dacomitinib blocks abnormal EGFR-family growth signals in selected lung cancers. Treatment aims to control advanced cancer and delay progression.

EGFR-FAMILY TARGETED THERAPY

Dacomitinib

Blocks abnormal growth signals

Activating EGFR mutation identified → Growth signalling blocked → Cancer growth slowed

Dacomitinib is an irreversible EGFR-family tyrosine kinase inhibitor. It interrupts signals involved in cancer-cell growth; the tumour mutation test and treatment setting determine whether it is suitable.

Report diarrhoea, rash, breathing changes and eye symptoms promptly.

Your treatment aim

Which setting?

NCCP 00565a covers first-line treatment of adults with locally advanced or metastatic non-small-cell lung cancer (NSCLC) with an activating EGFR mutation. This is daily dacomitinib alone; it is not an adjuvant regimen or a chemotherapy combination.

NCCP 00565 v3 · official regimen · EMA Vizimpro product information (updated 17 December 2024)

Duration

The usual starting dose is 45 mg by mouth once daily, continuously without a planned rest week. Use the dose on your prescription; some patients start lower because of liver function. Treatment continues while it helps and side effects remain acceptable.

What studies tell us

In a study of previously untreated advanced lung cancer with common EGFR mutations, dacomitinib delayed progression and improved overall survival compared with gefitinib. The study excluded people with a history of brain or other central nervous system metastases. It did not compare dacomitinib with every treatment available today. Group results cannot predict your own outcome.

Your own decision

Ask how the study population matches your situation and discuss alternatives, harms and remaining uncertainty.

ARCHER 1050 primary PFS: Wu et al., Lancet Oncology 2017 · ARCHER 1050 extended OS: Mok et al., Targeted Oncology 2021

Evidence for this treatment setting

The mutation and prior treatment matter.

Study groups cannot predict your individual outcome or compare treatments from different trials.

When to get help

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Call urgently for new or worsening breathlessness, cough or fever; severe diarrhoea, inability to drink or much less urine; severe or peeling skin reactions; or yellow skin/eyes. Eye pain, light sensitivity or blurred vision needs prompt assessment. Severe breathing difficulty, collapse or swelling of the tongue/throat needs emergency help. Use your unit's temperature and 24-hour contact instructions.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

SACTCHECK · CHAPTER 02

Your schedule, tests and appointments

Dacomitinib monotherapy · NCCP 00565

BASELINE → TREATMENT → FOLLOW-UP

Treatment and monitoring over time

Relative stages, not equal lengths of time or booked appointments. Daily tablets continue between reviews unless the team tells you to hold. Confirm who arranges and reviews every test.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00565 v3 · official regimen · EMA Vizimpro product information (updated 17 December 2024)

THE WHOLE TREATMENT JOURNEY

Dacomitinib monotherapy: treatment and checks

Your treatment pattern

Daily oral treatment without a rest week

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

EGFR-mutated advanced NSCLC · first-line monotherapy

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before the first dose	Early treatment review	Ongoing daily treatment	Benefit / toxicity review	Treatment change or completion
Treatment NCCP 00565 · SMPC	Confirm indication and prescribed dose	Daily dacomitinib; no rest week	Continue the prescribed dose	Review benefit, harms and dose	Agree the next treatment or follow-up
Blood tests NCCP 00565 · SMPC	Blood count, kidney, liver and bone profile	Repeat at locally agreed reviews	Continue regular tests	Extra tests for dehydration or abnormalities	Follow unresolved abnormalities
Early bowel and skin review NCCP 00565 · SMPC	Arrange symptom medicines and skin plan	Diarrhoea can start in the first 2 weeks	Report symptoms before the next visit	Team checks duration, grade and recovery	Continue support if symptoms persist
Symptoms and medicines NCCP 00565 · SMPC	Check symptoms, medicines and contact plan	Report early bowel/skin symptoms	Review adherence, supply and interactions	Confirm recovery before any restart	Continue advice for unresolved symptoms

Care / check	Before the first dose	Early treatment review	Ongoing daily treatment	Benefit / toxicity review	Treatment change or completion
Cancer assessment NCCP 00565 · SMPC	Confirm mutation, stage and previous treatment	Clinical review	Scans per Consultant plan	Discuss response or progression	Agree follow-up or next treatment

What each check is for

1



Confirm eligibility and the prescription

Before the first dose

The mutation and treatment setting determine applicability.

Ask / act: Document exact indication, dose and initial investigations.

2



Early bowel and skin review

From the first dose and early reviews

Early support can prevent dehydration and uncontrolled skin toxicity.

Ask / act: Record grade, duration, supportive treatment and recovery; apply medicine-specific thresholds.

3

New respiratory symptoms

At any time

Possible interstitial lung disease (ILD) requires urgent investigation.

Ask / act: Arrange urgent assessment and an authorised medicine plan.

4



Benefit and next-plan review

At planned cancer assessments or significant toxicity

Continuous treatment is not an unlimited automatic prescription.

Ask / act: Agree continuation, hold, restart, change or follow-up.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Checks before and during treatment

Tests and symptoms guide safe treatment decisions.

Before starting

Confirm the activating EGFR test, first-line treatment setting and prescription. Check blood count, kidneys, liver and bone profile, and arrange baseline staging scans. Review lung symptoms, bowel and skin health, all medicines and pregnancy precautions.

During treatment

NCCP requires regular blood count, kidney, liver and bone tests but does not specify a fixed interval; your team sets appointments. Diarrhoea often starts early, so keep the symptom plan ready from the first dose. Scans follow your Consultant's plan.

Taking dacomitinib

Follow your individual prescription.



Daily tablets

Take at approximately the same time each day with or without food and swallow with water. If you miss a dose or vomit, do not take a replacement or extra dose. Take your next prescribed dose at the usual time the following day.

Dose changes

The defined daily dose levels are 45 mg, 30 mg and 15 mg. After side effects, the team decides whether and when to restart and which dose applies. Do not select a dose or add a rest week yourself.

Practical treatment details

Supplies and visits

Arrange the next prescription before tablets run out.

The oral regimen itself does not require an infusion line or pump.

SACTCHECK · CHAPTER 03

Symptoms and supportive care

Dacomitinib monotherapy · NCCP 00565

Daily tablets, checks and decisions

No dates or personal information are needed. Confirm your own appointments with the team.

Treatment or check	Timing in this view
Preparation	**Before the first dose** Confirm the activating EGFR test, indication, prescription and baseline tests.
Daily oral treatment	**Early treatment review → Ongoing daily treatment → Benefit / toxicity review** The usual starting dose is 45 mg by mouth once daily, continuously without a planned rest week. Use the dose on your prescription; some patients start lower because of liver function. Treatment continues while it helps and side effects remain acceptable.
Tests and symptoms	**Early treatment review → Ongoing daily treatment → Benefit / toxicity review → Treatment change or completion** NCCP requires regular blood count, kidney, liver and bone tests but does not specify a fixed interval; your team sets appointments. Diarrhoea often starts early, so keep the symptom plan ready from the first dose. Scans follow your Consultant's plan.
Next treatment plan	**Treatment change or completion** Review response, side effects and whether to continue, change or stop treatment.

Take at approximately the same time each day with or without food and swallow with water. If you miss a dose or vomit, do not take a replacement or extra dose. Take your next prescribed dose at the usual time the following day.

Relative stages, not equal lengths of time or booked appointments. Daily tablets continue between reviews unless the team tells you to hold. Confirm who arranges and reviews every test.

> Columns show sequence, not equal durations. These are reference milestones, not booked appointments. Symptom bands do not predict severity or a safe time to ignore symptoms.

Living with treatment

Report symptoms early and keep contact details available.

Diarrhoea and fluids

Keep prescribed anti-diarrhoea medicine available from the first dose. Start your agreed plan at the first loose stool, keep drinking if able and report symptoms early. Your team may advise continuing symptom medicine until loose stools have stopped for 12 hours. Severe diarrhoea, weakness or reduced urine needs urgent advice.

Skin, nails and mouth

Use the agreed EGFR-inhibitor skin-care and mouth-care plan. Emollients and sun protection may help; report rash, painful nails or mouth sores early. The team decides whether creams, antibiotics or other treatment are needed.



Nausea medicines

Dacomitinib has minimal-to-low oral emetic risk. Routine infusion premedication is not required. Agree any as-needed anti-sickness medicine with the team, after checking interactions and your medical history.

NCCP 00565 v3 · official regimen · NCCP systemic anticancer therapy (SACT) antiemetic guidance V6 (2025), oral tables · NCCP NCIS antiemetic medicines V8 (2025); parenteral scope



Medicine interactions

Acid-suppressing medicines can reduce dacomitinib exposure. Proton pump inhibitors should be avoided if possible; ask pharmacy about alternatives. NCCP and current product information give different timing before H2 blockers, so obtain a written individual schedule. CYP2D6 medicines also need pharmacy review. Do not change medicines yourself.

Pregnancy, fertility and breastfeeding

Avoid pregnancy. Women who could become pregnant need adequate contraception during treatment and for at least 17 days after the final dose. Do not breastfeed during treatment. Discuss fertility and future pregnancy plans before starting.

When to get urgent help

Do not wait for the next appointment.

Call urgently

Call urgently for new or worsening breathlessness, cough or fever; severe diarrhoea, inability to drink or much less urine; severe or peeling skin reactions; or yellow skin/eyes. Eye pain, light sensitivity or blurred vision needs prompt assessment. Severe breathing difficulty, collapse or swelling of the tongue/throat needs emergency help. Use your unit’s temperature and 24-hour contact instructions.

When you call

Give the medicine name, last dose and symptoms. Tell the team if you can drink, pass urine and manage usual activities. Do not delay calling while gathering information.

When treatment changes

The team authorises continuation, interruption, restart or stopping.

Before restarting

Confirm symptom recovery, blood results, the prescribed dose and the next review.

When treatment ends

Agree follow-up, unresolved symptom care and reproductive precautions.

Practical support

Diarrhoea and fluids

Skin, nails and mouth

Nausea medicines

Medicine interactions

Pregnancy, fertility and breastfeeding

Taking tablets / attending treatment

Questions to ask

Is there a planned week off?

No. These tablets are continuous daily treatment. A hold for side effects is a clinical decision.

Can I adjust the dose myself?

No. Keep the prescription and contact plan. Your team authorises every hold, restart or dose change.

Do I need all the tests every day?

No. Your team schedules tests and reviews. Contact them between appointments if symptoms occur.

Explain the plan back: Can you explain how to take your tablets, what to do after a missed or vomited dose, which tests are next and which symptoms mean you should call today?

Which treatment this guide covers NCCP 00565a covers first-line treatment of adults with locally advanced or metastatic non-small-cell lung cancer (NSCLC) with an activating EGFR mutation. This is daily dacomitinib alone; it is not an adjuvant regimen or a chemotherapy combination.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Dacomitinib alone · daily oral treatment

Active medicines: dacomitinib

Emetogenic risk: Minimal to low (oral)

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Keep prescribed anti-diarrhoea medicine available from the first dose. Start your agreed plan at the first loose stool, keep drinking if able and report symptoms early. Your team may advise continuing symptom medicine until loose stools have stopped for 12 hours. Severe diarrhoea, weakness or reduced urine needs urgent advice.
- Use the agreed EGFR-inhibitor skin-care and mouth-care plan. Emollients and sun protection may help; report rash, painful nails or mouth sores early. The team decides whether creams, antibiotics or other treatment are needed.
- Dacomitinib has minimal-to-low oral emetic risk. Routine infusion premedication is not required. Agree any as-needed anti-sickness medicine with the team, after checking interactions and your medical history.
- Acid-suppressing medicines can reduce dacomitinib exposure. Proton pump inhibitors should be avoided if possible; ask pharmacy about alternatives. NCCP and current product information give different timing before H2 blockers, so obtain a written individual schedule. CYP2D6 medicines also need pharmacy review. Do not change medicines yourself.
- Avoid pregnancy. Women who could become pregnant need adequate contraception during treatment and for at least 17 days after the final dose. Do not breastfeed during treatment. Discuss fertility and future pregnancy plans before starting.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

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NCCP 00565 · VERSION 3

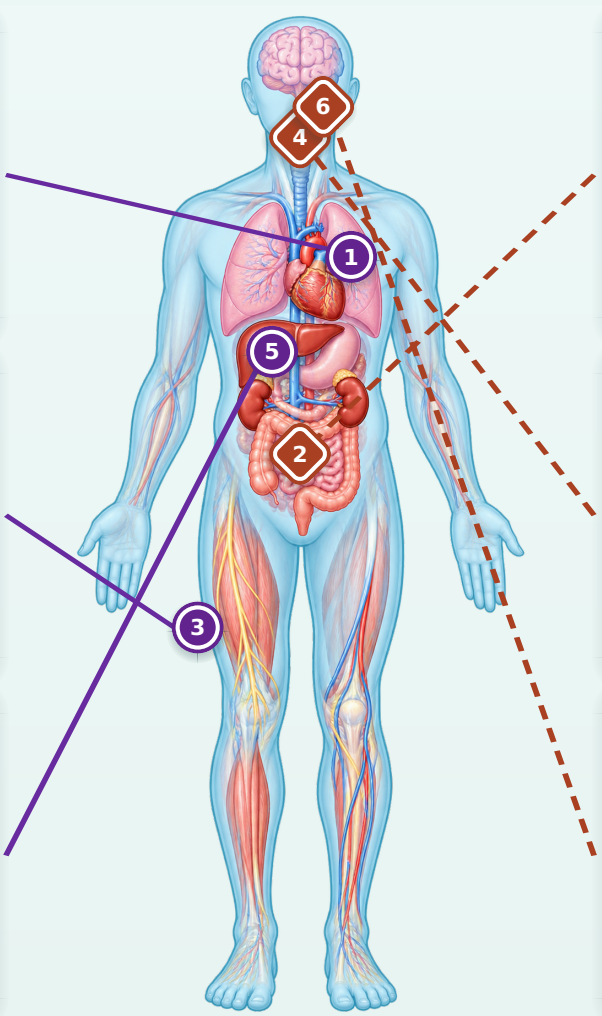
Dacomitinib monotherapy

The usual starting dose is 45 mg by mouth once daily, continuously without a planned rest week. Use the dose on your prescription; some patients start lower because of liver function. Treatment continues while it helps and side effects remain acceptable.

Oral targeted therapy · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 New cough or breathlessness

dacomitinib

Possible lung inflammation needs urgent assessment.

2 Diarrhoea and dehydration

dacomitinib

Diarrhoea can become serious quickly and may affect the kidneys.

3 Rash, nail changes or peeling

dacomitinib

Early treatment can help; severe reactions need urgent review.

4 Mouth soreness

dacomitinib

Report difficulty eating or drinking.

5 Yellow eyes or skin

dacomitinib

Liver injury may need blood tests and treatment review.

6 Eye pain or blurred vision

dacomitinib

Eye inflammation or keratitis needs assessment.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call urgently for new or worsening breathlessness, cough or fever; severe diarrhoea, inability to drink or much less urine; severe or peeling skin reactions; or yellow skin/eyes. Eye pain, light sensitivity or blurred vision needs prompt assessment. Severe breathing difficulty, collapse or swelling of the tongue/throat needs emergency help. Use your unit’s temperature and 24-hour contact instructions. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

SACTCHECK · CHAPTER 04

Your record and contact card

Dacomitinib monotherapy · National Cancer Control Programme (NCCP) 00565

Bring this to reviews. Record symptoms, treatment changes and questions when helpful. You do not need to complete a daily diary or calculate a toxicity grade.

The four sheets are reusable review periods, not a prescribed number of cycles. Follow the treatment and rest days agreed with your team; do not calculate a restart from this sheet.

These patient-language prompts are not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) assessment. Urgent symptoms override the form.

Review period / visit: _____ Next review: _____

Medicine-specific symptom	No concern	Present / manageable	Affects sleep or daily activities — contact team	When / what changed?
Diarrhoea	☐	☐	☐	
Skin / nail changes	☐	☐	☐	
Mouth soreness / ulcers	☐	☐	☐	
New cough or breathlessness	☐	☐	☐	
Eye pain or blurred vision	☐	☐	☐	
Fatigue	☐	☐	☐	

Checks: expected, booked and reviewed are different

Test / review	Due window	Who books it?	Who reviews the result?	Outcome / next action
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____

Missed tablets / injection changes / questions / support needed:

Call urgently for new or worsening breathlessness, cough or fever; severe diarrhoea, inability to drink or much less urine; severe or peeling skin reactions; or yellow skin/eyes. Eye pain, light sensitivity or blurred vision needs prompt assessment. Severe breathing difficulty, collapse or swelling of the tongue/throat needs emergency help. Use your unit's temperature and 24-hour contact instructions. Emergency symptoms: 112 / 999.

Carry your treatment details

Which treatment this guide covers NCCP 00565a covers first-line treatment of adults with locally advanced or metastatic NSCLC with an activating EGFR mutation. This is daily dacomitinib alone; it is not an adjuvant regimen or a chemotherapy combination.

Print, cut out and fold. Follow your treating hospital’s contact plan.

SACTCHECK · TREATMENT CARD

Dacomitinib monotherapy

NCCP 00565 v3

The usual starting dose is 45 mg by mouth once daily, continuously without a planned rest week. Use the dose on your prescription; some patients start lower because of liver function. Treatment continues while it helps and side effects remain acceptable.

Prescriber / next review: _____

Contact plan

Oncology team: Insert numbers here · Use your hospital’s opening hours.
Out of hours: Insert numbers here.
Evenings / overnight: agreed out-of-hours service or emergency department.
Emergency: 112 / 999.

Own hospital / contact: _____

Call urgently for new or worsening breathlessness, cough or fever; severe diarrhoea, inability to drink or much less urine; severe or peeling skin reactions; or yellow skin/eyes. Eye pain, light sensitivity or blurred vision needs prompt assessment. Severe breathing difficulty, collapse or swelling of the tongue/ throat needs emergency help. Use your unit’s temperature and 24-hour contact instructions.

https://sactcheck.com/patient/00565/ · no patient information encoded.

SACTCheck · Developed by Dr Paul O’Brien · v0.84.25 · Sources checked 2026-10-29
Source reconciled; independent clinical, pharmacy and patient/PPI review pending

SACTCHECK · CHAPTER 05

Sources and further information

Dacomitinib monotherapy · National Cancer Control Programme (NCCP) 00565

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00565 v3 · official regimen**
   https://healthservice.hse.ie/documents/6894/565_V3_Dacomitinib.pdf
2. **EMA Vizimpro product information (updated 17 December 2024)**
   https://www.ema.europa.eu/en/documents/product-information/vizimpro-epar-product-information_en.pdf
3. **NCCP V6 oral minimal-low: individual as-required antiemetics; NCIS V8 is parenteral and does not prescribe an oral course**
   https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_systemic anticancer therapy (SACT)_Induced_Nausea__pWgDCrm.pdf
4. **NCCP NCIS antiemetic medicines V8 (2025); parenteral scope**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
5. **ARCHER 1050 primary PFS: Wu et al., Lancet Oncology 2017**
   https://pubmed.ncbi.nlm.nih.gov/28958502/
6. **ARCHER 1050 extended OS: Mok et al., Targeted Oncology 2021**
   https://pubmed.ncbi.nlm.nih.gov/33331989/